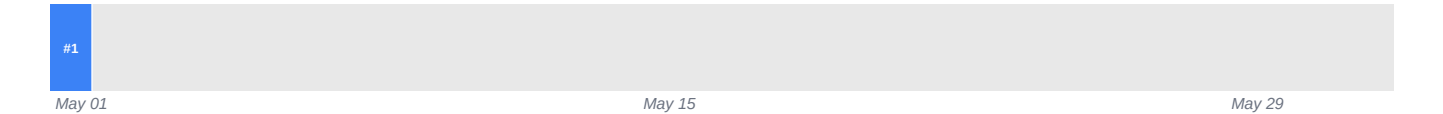


Patient ID: ThornburgPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 743/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 32 days from May 01 to Jun 01

■ HR ■ Breathing



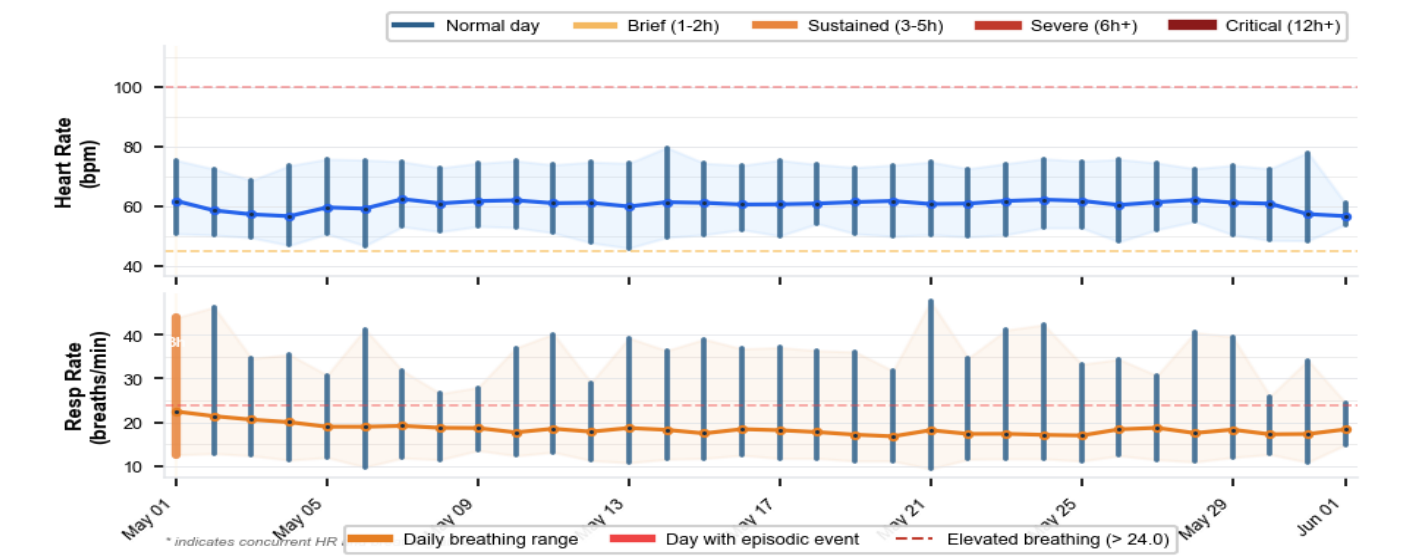
Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
3	3h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Elevated Breathing	29 brpm	3h	1h	3	24 brpm	May 01

Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

- Clinical Pattern Observations:
- 1. **Clustered pattern:** Episodic events on 1 of 32 days (3%).
 - 2. **Nocturnal pattern:** 2 of 3 eligible episodes during evening or night hours.



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Low HR < 45 bpm

Elevated HR > 95 bpm

High HR > 100 bpm

Very High HR > 110 bpm

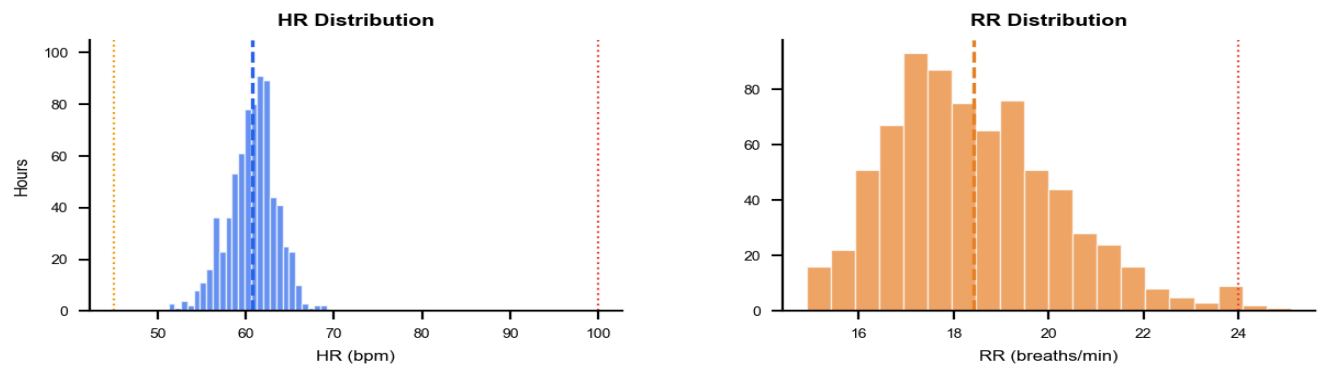
Elevated Breathing > 24 brpm

High Breathing > 30 brpm

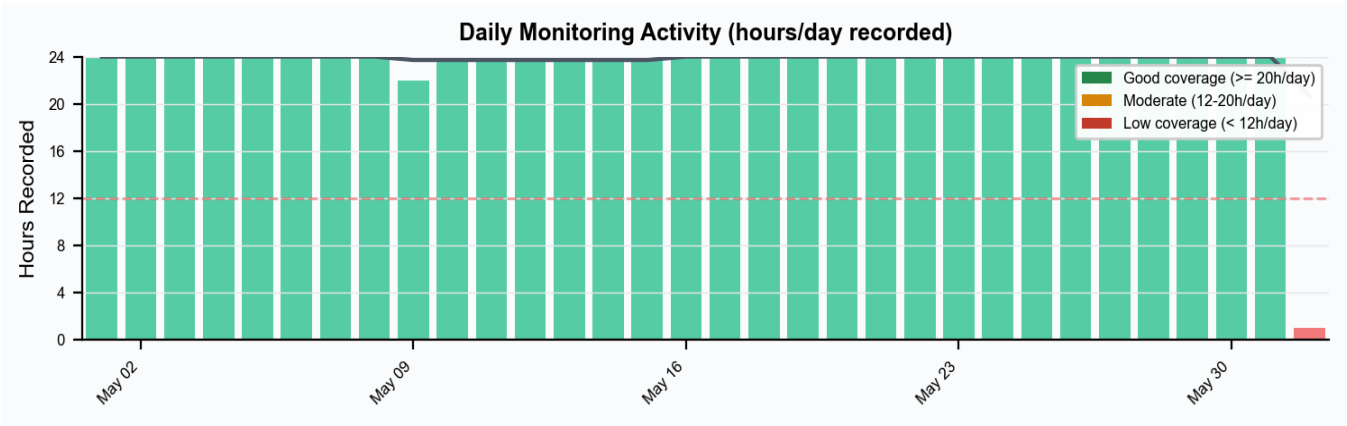
Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.